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Name: عائشه ابراهيم محمد مرسى National ID: 25301170102961	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدى و تحتاج الى (Phesgo (Subcutaneous trastuzumab and pertuzumab

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بمناقضة تحقيق الشخصية



عائشة
ابراهيم محمد مرسى
٦٥ ش محمد الصغير
مصر القديمة - القاهرة

٢٥٣.١١٧.١.٢٩٦١

J10407015

١٠ م ٣

فيسو

٢٠٢٢/٠٣ ٢٥٣.١١٧.١.٢٩٦١

حاصلة على دبلوم صناعات

انشى مسلعة متروجة
محمد عبدالمجيد خليل ابراهيم
البطاقة سارية حتى ٢٠٢٩/٠٣/٢٤




05/25/2026

MDT FORM

Name:	Eaylshah Ibrahim Mohampud Morse	MRN:	6287
Age:	73 Y	ID:	25301170102951
Menopause Status:	postm	Phone:	01223362648
NRS SCORE:	3	Marital Status:	
BMI:		Family History:	
CO Morbidity:	parkinsonism	Reason Of MDT:	

Clinical Data:

BX: 216

ER 8/8 PR 0/8 HER 3+ KI67

Sonomammography:

Mets Workup:

History:

Full "T" CT BREAST CANCER CT4B N2 M0 BR8/8 P-VE HER2 3+
pt received 15 weeks taxol carboplatin 4 phesgo chemotherapy was delayed multiple times bac of grade 3 neutropenia inspite of dose reduction and

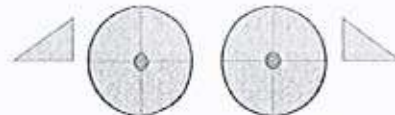
Recommendations:

MDT Outcome & Plan

LT MRM

R

L



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ محمد عبد الحليم					

Name: عائشه ابراهيم محمد مرسى		
MR. : 6287		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض	
بقسم:	رقم طبي:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :	
ويعد الكشف والفحوصات والمتابعة وجد انه		
POSTM LEFT BREAST CANCER CT4 N2 M0 ER8/8 PR-VE HER2 3+ FOR W8 NEOAD TAXOL CARBO POSTM LEFT BREAST CANCER CT4B N2 MO ER8/8 P-VE HER2 3+ SA1.7 EF 67% FOR NEOAADJ W10100 MG WITH DR AND C2 PHESGO WITH PROPHYLACTIC NEUPOGEN PROPHYLACTIC EVERY WEEK BEC OF PROLONGED NEUTROPENIA W11 TAXOL CARBO TAXOL 100 CARBO 1.5 AUC S.A 1.7 TLC 9.4 1 AMP PROPHYLACTIC NUBOGEN C3 PHESGO 600 S.C AT 12/2/2026 ECHO 21/10 EF67% W12 TC 5/2 PHARMACY CBC W13 TC 12/2 PHARMACY + C3 PHESGO 600 S.C CBC pt has grade 3 diarrhea grade 3 om to postpone 1 week .last chemo on 29/1.for c3 phesgo ,us breast supp tt and assess ps next time and consider further dr bec pt had grade 3 diarrhea for further dr 10% and assess ps next week .us breast12/2 left ax ln largest 7.5x5mm . on exam mild edema .for w12 taxol 90 mg and carboplatin auc 1.5 bec pt is so frail and assess ps next week . on exam no breast edema or breast or ax ln for w12 taxol 90 carbopl auc 1.5 ef 70% 21/2/2026 with prophylactic neupogen pt is neutropenic for c4 phesgo and postpone 1 week and consider proph neupogen every week today w13 taxol 90 carbopl auc 1.5 , week 14 (pharmacy) , week 15 taxol (pharmacy + cbc at week 15 + nebogen) phesgo 26/03/2026 (pharmacy) today w15 taxol 90 carbopl auc 1.5 (NACT) , week 16 taxol carbo (pharmacy +cbc) , week 17 taxol, carbo (pharmacy + cbc) TLC 27/12 >> labs accepted Pt received GcSF 3 ampl last week pt has grade 2 pn grade 2 onychomycosis tlc 2.3/0.7 to postpone 1 week for c5 phesgo after being available and neupogen .on exam no bresast edema o0n exam no palpable breast masses grade 2 pn improved for w16 taxol 90 carbopl auc 1.5 and c5 phesgo and 2 proph neupogen on exam newly developped rt breast redness for us to postpone cycle 1 week bec pt is neutropenic for c5 phesgo and new us breast for mdt for surgery (intolerability of chemo and suspicious local pd)		
تحريرا في ,		
التاريخ :	19-09-2026	الوقت :
الطبيب المعالج :	dina makram	
يعتمد ,		

عائشه ابراهيم محمد مرسى - (MR 6287 (continued, page 2/2

mdt pending us breast 10/5/2026 diffuse edema left ax ln largest 8x4.5 mm on exam mil rt breast redness for w17 taxol 90 carbopl auc 1.5

mdt for left mrm pt is neutriopenic to omit w18 and for c5 phesgo ef 2/2026 64

patient undewent left MRM on 15/8/2026: ypT0N0 PCR LNs 0/25 , no MDT post op yet and did not go to surgery to be transferred to surgery clinic first then for MDT for adjuvant ttt.

Patient Name	عائشة إبراهيم محمد	Age	72
Sex	F	Special habits	
Family History		Comorbidity	HTN + splenectomy + parkinsonism
Surgical History		Telephone	01223362648
Menopausal Status	post	Marital Status	
ID	25301170102961	Medical ID	2833/2025
	14-10-2025	Reason for MDT	new

Medical Report

Pathology: left axillary L.N → metastatic cancer

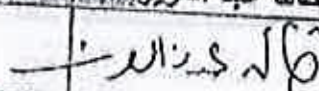
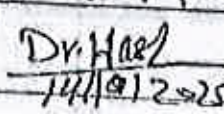
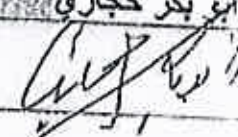
ER8/8 PR 0/8 HER2 +3 KI6730%

T4B N2 M0

- Presentation :
- S.M. : diffuse edema pattern of the left breast with retroareolar linear and grouped microcalcification + pathological axillary L.N
- Metsworkup : CT CAP → free of mets
- Recommendations : bone scan → requested

MDT
DECISION

For: 6xTCHP followed by left MRM

Name	د/ خالد عبد العزيز	د/ هاشم عبد الرحمن	د/ ابو بكر حجازي
Signature		 14/10/2025	

Patient Name : الأستاذة / عائشة إبراهيم محمد مرسى
Patient No. : 25301170102961
Path No. : 2908/25
Sex : Female
Age : 72 Y
Referring Doc :

Request No. : 13001479
Request Date : 24/09/2025
Verification Date : 01/10/2025
Financial Category : جوفات روسي - مبادره
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Diffuse edema pattern of the left breast with retroareolar linear and grouped microcalcifications as well as ipsilateral pathological axillary lymph nodes.
Nature of Specimen: Left axillary lymph node, Tru-cut biopsy.

Gross Examination:

- Four fragmented grey white tissue cores, the longest was 0.8 cm long. Totally submitted.

Technique:

- Sections prepared from paraffin block for Immune-staining using Bench Mark ULTRA system for:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & Ki67 clone (30-9).

Microscopic Examination:

* Histopathological examination reveals cores of lymphoid tissue showing infiltration by tumor formed of malignant epithelial cells arranged in groups with moderate tendency to tubular formation.

* IPT (Positive control showed appropriate reactivity):

- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 30%.

Diagnosis:

Left axillary lymph node, Tru-cut biopsy:
- Metastatic carcinoma, Likely of breast origin.

IPT:

- ER: Positive (score 8/8).
- PR: Negative (score 0/8).

Verified by :

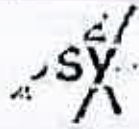
Dr. Duaa Mohamed

Printing Date : 01/10/2025 13:16:18

Page 1 of 2

Printing By : Mrs. Mayada Hegazy

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name : الأستاذة / عائشة إبراهيم محمد مرسى
Patient No. : 25301170102961
Path No. : 2908/25
Sex : Female
Age : 72 Y
Referring Doc :
Request No. : 13001479
Request Date : 24/09/2025
Verification Date : 01/10/2025
Financial Category : جرماتك رومى - مباله
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

- Her2: Positive (score 3+).
- Ki67: 30%.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Duaa Mohamed.

Verified by :

Dr. Duaa Mohamed

Printing Date : 01/10/2025 13:16:18

Page 2 of 2

Printing By : Mrs. Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]



Name: Aalesha Ibrahim Mohamed
Referred by: Prof.

ID: 4894453

REPORT

Right breast full field digital mammography revealed:

Patient under NAC for left breast cancer.

- Parenchymal tissues condensation in the UOQ of the breast with predominant nodular densities.
- No dominant masses, areas of architectural distortion or pathological micro-calcifications elicited in the breast.
- Few benign calcific foci are seen in the breast.
- Normal skin thickness and contour of the breast as well as its nipple areolar complex.

Complementary high resolution breast US revealed:

- Normal parenchymal echogenic pattern.
- No solid masses or cystic lesions seen.
- No ductectasia.
- Non specific likely right axillary lymph nodes.

IMPRESSION:

- Few benign calcifications. BIRADS 2.
- For regular follow up.

Prof. Dr. Maha Helal, MD



Name: Aa'esia Ibrahim Mohamed
Referred by: Prof.

ID: 4829839

Date: 20/09/2025

Thanks for reference

Left Digital soft tissue mammography & high resolution breast sonography & Elastography revealed :

- Scattered fibroglandular densities (ACR-B).
- Diffuse edema pattern of the left breast in form of skin thickening reaching about 6 mm, echogenic fat lobules, increased trabeculations and Interstitial edema.
- Retro areolar linear and grouped microcalcifications. By U/S showing retroareola altered parenchyma with no sizable solid masses.
- No cystic masses.
- Multiple pathologically enlarged axillary lymph nodes involving level I and II, some showing irregular cortical outline, largest measures 30x17 mm.

OPINION :

Diffuse edema pattern of the left breast with retroareolar linear and grouped microcalcifications as well as ipsilateral pathological axillary lymph nodes (BIRADS 4), for histopathological correlation.

Kindly accept my best regards
Dr. Salma Mostafa, MI

Defining BIRADS classification of breast lesions according to the American Collage of Radiology:

Category

Suspected Cancer Risk

BIRADS 0 : Non-informative study.	
BIRADS 1 : Normal study.	
BIRADS 2 : Benign findings.	0 %
BIRADS 3 : Probably benign.	0 %
BIRADS 4 : Probably malignant.	2 %
A : mild suspicion.	50 %
B : moderate suspicion.	
C : high suspicion.	
BIRADS 5 : Malignant.	98-100%
BIRADS 6 : Pathologically proven malignant lesion.	



Patient Name: Aa'esha Ibrahim Mohamed Morsi	Date : 07/10/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 72 Year(s)
Medical ID: 2833/2025	Pat. ID: 121984

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- A patient with reported history of left cancer breast.
- The left breast is partially scanned with the visualized portions showing skin thickening and heterogenous braet parenchyma for correlation with sonomammography.
- Multile enlarged left axillary and subpectoral lymph nodes are seen, largest measures about 2.6 cm in maximum diametr.
- Prominent retrocaval and rlgh axillary lymph node measuringhg 8 mm in diameter.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Left thyrold small hypodense exophytic nodule seen in antrlor mediastinum measuring about 1.4 cm.
- Bilateral elastofibroma dorsi.

II Abdomen & pelvis:

- The liver is of average size showing diffuse reduction of its parenchymal attenuation value denoting fatty infiltration. No definte enhancing focal lesions or contour irregularities.
- No Intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries



Patient Name: Aa'esha Ibrahim Mohamed Morsi	Date : 07/10/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 72 Year(s)
Medical ID: 2833/2025	Pat. ID: 121984

- The spleen is not visualized with a small soft tissue measuring 2 cm at its anatomical site? Splenule? Residual scarred splenic tissue..
- Normal size, parenchymal thickness & density of both kidneys (apart from left renal cortical cysts) with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta (apart from atheromatous calcification), I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Fat containing right inguinal hernia is seen.

***Scanned skeleton:** no definite worrisome lytic or sclerotic focal lesions.
Reduced height of L1 vertebral body is noted.

Opinion:

A patient with reported history of left breast cancer revealed:

- Left axillary and subpectoral lymphadenopathy, likely metastatic.
- Fatty liver.
- The spleen is not visualized with a small soft tissue measuring 2 cm at its anatomical site? Splenule? Residual scarred splenic tissue.
- Fat containing right inguinal hernia.
- Small left thyroid exophytic nodule.
- No evidence of pulmonary nodules, hepatic or osseous metastatic deposits.

Name : Aisha Ibrahim Mohammed Musa
Branch : Dokki
Ref.Physician Prof : Gostaf Roseh Hospital

Date : 13-10-2025 12:31
Medical No. : 224195
Ins.Company : تعاقد مستشفيات

Tc^{99m} MDPSKELETAL SCINTIGRAPHY

History

- A 72-year-old lady, with history of left breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

- No abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

ECHO REPORT

NAME: عائشة الوائلي

DATE: 21 / 10 / 2025

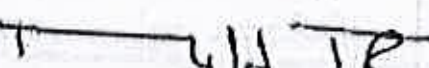
ID NUMBER: ٢٨٧٢ / ٢٥

SEX: ♀ AGE: 72y

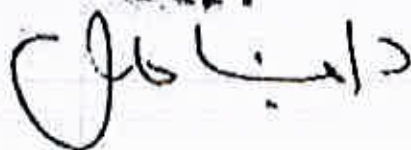
EF	67.1	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.5	3.5-5.8cm	AO.R	2.5	3-3.7cm
LVESD		2-4cm	LA	3.7	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions LV Wall Thickness 

LV Diastolic Function DD grade II

SWMA LA and Ao Root Dimensions Mitral Valve Aortic Valve Tricuspid Valve Pulmonary Valve Right Side and Systolic Function Intracardiac Masses and Effusion 

DR.





رسوم الإعلام 211 جنينة

تاريخ الإصدار: 2025-9-22

جهة الإصدار: فرع القاهرة

التي من يهمة الأمر

الهيئة العامة للتأمين الصحي تعمل من خلال قوانين منظمة للاحتفاع الخاضعين لقانون التأمين
الاجتماعي رقم 79 لسنة 1975 المعدل بالقانون 25 لسنة 1977 "القوى العاملة والمعاشات
والارامل" وكذلك قانون 99 لسنة 1992 الخاص بطلاب المدارس وقانون رقم 86 لسنة 2012
الاطفال دون السن المدرسي" وقانون رقم 23 لسنة 2012 الخاص بالمرأة المعيلة.
وطبقا لقاعدة البيانات المرسلة من التأمينات الاجتماعية
وطبقا لقاعدة البيانات المرسلة من الضمان الاجتماعي

في البحث، تبين أن

المستفيد (ة) عائشة ابراهيم محمد مرسى بطاقة رقم 25301170102961

تأليف رقم: 111

وتطبقا الى خطاب من التأمينات الاجتماعية يفيد عدم الاحتفاع
وتعتبر من الفئات غير المتفقه بالتأمين الصحي صدر هذا الخطاب دون انفس مسؤولية على الهيئة
العامة للتأمين الصحي حاله خضوع المواطن لقانون التأمين الصحي الشامل
وشكرا

تاريخ الإصدار: 2025-9-22

الموظف المختص

محمود عبد الله



الإدارة المركزية للنظم المعلومات

محمود عبد الله

1780211



الخطاب صادر في لائحة لاؤلة التهر من تاريخ الإصدار الى ان حظه تغيير. هذا الى ان من قبل الهيئة العامة للتأمينات الاجتماعية أو
وزارة الصحة العامة أو وزارة الداخلية أو وزارة التعليم

الإدارة المركزية للنظم المعلومات والتحول الرقمي

8/26/26, 4:57 PM

code:02 - Name_A MDT - Name_E MDT

		مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية			
MDT FORM		08/26/2026			
Name: Eayishah Ibrahim Mohamoud Morse		MRN: 6287			
Age: 74 Y		ID: 25301170102961			
Menopausal Sta: POST		Phone: 01223362648			
NRS SCORE:		Marital Status:			
BMI:		Family History:			
CO Morbidity: HTN		Reason Of MDT: POST OP			

Clinical Data:

BX: **R** **L**
ER: 8/8 **PR:** 0/8 **HER2:** 3 **KI67:**

Sonomammography:

Mets Workup:

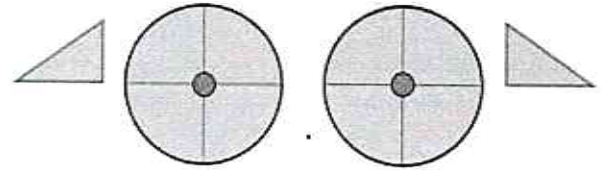
History:

POSTM LEFT BREAST CANCER CT4B N2 MO ER8/8 P-VE HER2 3+
 pt received 15 weeks taxol carboplatin 4 phesgo chemotherapy was delayed
 multiple times bec of grade 3 neutropenia inspite of dose reduction and proph
 neupogen for mdt bec of intolerance to chemotherapy and newly developed
 edema and reness (mild and localized)
 PTN underwent It MRM
 POP
 NO residual tumor
 LN 0/25
 ypT0,YPN0

Recommendations:

MDT Outcome & Plan

For PORT
 Continue Phesgo up to total 1 year
 Femara x 5-7 years + Zometa / 6 m



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. خالد عبدالعزیز	د. م. النوراني				

مستشفى
روسيه
القاهرة



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

مستشفى
المرسى
مصر

أبي
أ-ع7

08/26/2026

□

MDT FORM

Name:	Eyishah Ibrahim Mohamoud Morse	MRN:	6287
Age:	74 Y	ID:	25301170102961
Menopausal Ste	POST	Phone:	01223362648
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	HTN	Reason Of MDT:	POST OP

Clinical Data:

BX:

R

L

ER: 8/8

PR: 0/8

HER2: 3

KI67:

Sonomammography:

Mets Workup:

History:

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neupogen for mdt bec of intolerance to chemotherapy and newly developed
edema and reness (mild and localized)
PTN underwent lt MRM
POP

T N M

NO residual tumor
LN 0/25

ypT0, ypNO

Recommendations:

MDT Outcome & Plan

For PORT
Continue Phesgo up to total 1 year
Femara x 5-7 years + Zometa / 6 m

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
أ. د. محمد عبد الله	أ. د. محمد عبد الله				

أبي
أ-ع7

Patient Name	عائشة بن أحمد محمد	Case No.	341532
Sex/Age	Female 73 Y	Patient No.	24689
Registered Date	12/7/2026 10:55:41 AM	Referred By Dr.	Surgery
Validation Date	11/8/2026 03:25:46 PM	Branch	Gustave Roussy
Reporting Date	15/8/2026 11:29:54 AM	Pathology No.	2188/26

Pathology Report

Clinical Data: Left breast cancer, post-NACT.

Nature of the specimen: Left modified radical mastectomy.

Gross Examination:

- Gross Examination:**
- * Left modified radical mastectomy specimen measured 27x17x10 cm, with covering skin ellipse measured 25x15 cm. Nipple & areola looked unremarkable grossly.
 - * Serialing showed a rubbery grey, white tumor bed site measuring 13x8 cm.
 - * Axillary: fibro fatty tissue pieces entangling 25 lymph nodes, the largest measured 2x1x0.5 cm.
- * Block designation:**
- * 14 Blocks:**
- 1:6- T.B.S., 7- Deep margin, 8- N&A., 9:14- ALNs.
- Gross was done by; Dr. Aya Ezzat.

Microscopic examination:

- **Histological type:** No residual viable remnants of the previously diagnosed invasive tumor.
- **Tumor size:** Unclipped tumor bed site assessed grossly by (13 cm).
- **Ductal carcinoma in situ:** Absent.
- **Angioinvasion:** Absent.
- **Perineural invasion:** Absent.
- **Microcalcification:** Absent.
- **Nipple and areola:** Free.
- **Lymph node involvement:** Twenty-five lymph nodes free of metastatic deposits (0/25) with notable therapy effect.
- **Resection Margins:** Free deep margin.
- **Histologic response to therapy:** Pathological complete response.

Diagnosis:

Left Breast Cancer (Post-NACT); Left Modified Radical Mastectomy:

- No residual viable remnants of the previously diagnosed invasive tumor.
- Nipple and areola: Free.
- Deep margin: Free.



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Gustave Roussy International-Egypt Pathology Lab
جوستاف روصي الدولي للبحوث المرضية - مصر



Patient Name	عائشة أبو الفتح محمد	Case No.	341532
Sex/Age	Female 73 Y	Patient No.	24689
Registered Date	12/7/2026 10:55:41 AM	Referred By Dr.	Surgery
Validation Date	11/8/2026 03:25:46 PM	Branch	Gustave Roussy
Reporting Date	15/8/2026 11:29:54 AM	Pathology No.	2188/26

- Axillary lymph nodes: Twenty-five lymph nodes free of metastatic deposits (0/25) with notable therapy effect.
 - Histologic response to therapy: Pathological complete response.
- Stage: ypT0, ypN0.

Signature
Prof. Dr. Rana Elsaed.



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